



INTERIM NATIONAL COMMITMENTS AND POLICY INSTRUMENT 2025

for reporting period January - December 2024

Interim NCPI

Abbreviations

1HP	1 month of daily rifapentine plus isoniazid
3HP	3 months of weekly rifapentine plus isoniazid
3HR	3 months of daily rifampicin plus isoniazid
3TC	lamivudine
4R	4 months of daily rifampicin
6H	6 months of daily isoniazid monotherapy
6Lfx	6 months of daily levofloxacin monotherapy
9H	9 months of daily isoniazid monotherapy
ABC	abacavir
ATV/r	atazanavir/ritonavir
AZT	zidovudine
CAB-LA	long-acting injectable cabotegravir
CPR	C-reactive protein
CrAg	cryptococcal antigen
DPV-VR	dapivirine vaginal ring
DRV/r	darunavir/ritonavir
DTG	dolutegravir
DVR	dapivirine vaginal ring
EFV	efavirenz
EWI	early warning indicator
FDC	fixed-dose combination
FTC	emtricitabine
Global Fund	Global Fund to Fight AIDS, Tuberculosis and Malaria
HBV	hepatitis B virus
HIVDR	HIV drug resistance
IGRA	interferon-gamma release assay
LF-LAM	lateral flow urine lipoarabinomannan assay
LPV/r	lopinavir with a ritonavir boost
MNCH	maternal, newborn and child health
NCD	noncommunicable diseases
NCPI	National Commitments and Policy Instrument
NNRTI	non-nucleoside/nucleotide transcriptase inhibitors
NRTI	nucleoside reverse transcriptase inhibitor
PDR	pre-treatment drug resistance
PEP	post-exposure prophylaxis
PrEP	pre-exposure prophylaxis
RPR	rapid plasma reagin
TAF	tenofovir
TB	tuberculosis
TDF	tenofovir disoproxil fumarate
TPHA	<i>Treponema pallidum</i> hemagglutination assay
TPPA	<i>Treponema pallidum</i> particle agglutination assay
UNAIDS	Joint United Nations Programme on HIV/AIDS
UNFPA	United Nations Population Fund
VDRL	venereal disease research laboratory
WHO	World Health Organization

Interim National Commitments and Policy Instrument

* The guidelines for the NCPI define the terms marked with an asterisk (*).

1. Combination prevention for all

- Reduce new HIV infections to under 370 000 by 2025.
- Ensure that 95% of people at risk of HIV infection—within all epidemiologically relevant groups, age groups and geographic settings—have access to and use appropriate, prioritized, person-centred and effective combination prevention options.
- Reduce the number of new HIV infections among adolescent girls and young women to below 50 000 by 2025.
- Ensure availability of pre-exposure prophylaxis (PrEP) for 10 million people at substantial risk of HIV and post-exposure prophylaxis (PEP) for people recently exposed to HIV by 2025.
- 95% of people within humanitarian settings at risk of HIV use appropriate, prioritized, people-centred and effective combination prevention options.

Pre-exposure prophylaxis

1. Do your country's national guidelines recommend any of the following pre-exposure prophylaxis (PrEP) modalities or products (select all that apply)?

- ☒ ☐ Oral PrEP containing tenofovir (TDF)
- ☐ Dapivirine vaginal ring (DPV-VR or DVR)
- ☐ Long-acting Injectable cabotegravir (CAB-LA)
- ☐ No PrEP modalities/products are recommended in the national guidelines

1.1 To which populations is PrEP provided under the national guidelines (select all that apply)?

- ☒ ☐ Gay men and other men who have sex with men
- ☒ ☐ Sex workers
- ☐ ☒ People who inject drugs
- ☒ ☐ Transgender people
- ☒ ☐ Serodiscordant couples
- ☒ ☐ Young women (aged 18–24 years)
- ☒ ☐ Adolescents (aged <17 years)
- ☐ ☐ People in prisons and other closed settings
- ☐ ☐ Pregnant and breastfeeding women
- ☒ ☐ People who request PrEP
- ☐ ☐ Other (please specify): _____
- ☐ ☐ No national PrEP guidelines have been developed

1.2 Who has the authority to provide PrEP in your country (select all that apply)?

- ☒ ☐ Doctors
- ☐ ☐ Clinical officers
- ☐ ☐ Nursing cadre (e.g. midwives, nurse practitioners, registered nurses)
- ☐ ☐ Pharmacists
- ☐ ☐ Community health workers
- ☐ ☐ Other (please specify): _____
- ☐ ☐ No provider has authority to provide PrEP in the country

1.3 Is PrEP available through any of the following in your country (select all that apply)?

- ☒ ☐ Public health-care facilities
- ☒ ☐ Community-based distribution (including mobile services)
- ☐ ☐ Pharmacies (stand-alone, including online)
- ☐ ☐ Private health-care providers
- ☐ ☐ The internet
- ☐ ☐ Research sites
- ☒ ☐ Other (please specify): **Civil Service Organizations**
-

Post-exposure prophylaxis

2. **Do your country's national guidelines recommend post-exposure prophylaxis (PEP) for the following groups (select all that apply)**
- ☒ Exposure related to health care (occupational exposure)
- ☒ Exposure related to sexual or gender-based violence
- ☐ Any other exposure
- 2.1 **Who has the authority to provide PEP in your country (select all that apply)?**
- ☒ Doctors
- ☐ Clinical officers
- ☐ Nursing cadre (e.g. midwives, nurse practitioners, registered nurses)
- ☐ Pharmacists
- ☐ Community health workers
- ☐ Other (please specify): _____
- 2.2 **Is PEP available through any of the following in your country (select all that apply)?**
- ☒ Public health-care facilities
- ☐ Community-based distribution (including mobile services)
- ☐ Pharmacies (stand-alone, including online)
- ☒ Private health-care providers
- ☐ The internet
- ☐ Research sites
-

Voluntary medical male circumcision

Please note that these questions are only asked of 15 countries with high HIV prevalence, low levels of male circumcision and generalized heterosexual epidemics: Botswana, Eswatini, Ethiopia, Kenya, Lesotho, Malawi, Mozambique, Namibia, Rwanda, South Africa, South Sudan, Uganda, the United Republic of Tanzania, Zambia and Zimbabwe.

3. **What medical male circumcision methods are recommended/approved by the national programme (select all that apply)?**
- ☐ **N/A** Conventional surgical methods (dorsal slit, forceps guided, sleeve resection)
- ☐ World Health Organization (WHO) prequalified device method approved for use
-

Condoms

4. **Has the national need for condoms been estimated?**
- ☒ Yes
- ☐ No
- 4.1 **If yes, what is the estimated number of condoms needed? 18.7416 million**
- 4.2 **If yes, for what year is the condom needs estimate? 2017**
- 4.3 **If yes, what method was used to estimate the number of condoms needed (select all that apply)?**
- ☒ General population (condoms per sexually active man/year)
- ☒ Historical (same as last year + population growth)
- ☐ Budget-driven (based on what can be bought)
- ☒ Demand-based (based on past condom usage rates, such as using the GOALS model)
- ☐ Capacity-based (how many can be supplied and distributed with current capacity)
- ☒ Part of family planning commodity needs estimates
- ☐ "Total universe of need" approach
- ☐ UNFPA/UNAIDS Condom Needs and Resource Requirement Estimation Tool
- ☐ Other (please specify): _____
-

5. Have there been condom stock-outs* in the past 12 months?

5.a National stock-outs

- ☐ Yes
☒ No

5.b Local stock-outs

- ☒ Yes
☐ No
-

2. 95–95–95 for HIV testing and treatment

- Reduce annual AIDS-related deaths to under 250 000 by 2025.
 - Ensure that 34 million people are on HIV treatment by 2025.
 - Achieve the 95–95–95 testing, treatment and viral suppression targets within all demographics, groups and geographic settings, including children and adolescents living with HIV:
 - 95% of people living with HIV know their HIV status.
 - 95% of people who know their HIV-positive status are accessing treatment.
 - 95% of people on treatment have suppressed viral loads.
 - Ensure that 90% of people living with HIV receive preventive treatment for tuberculosis (TB) by 2025.
 - Reduce TB-related deaths among people living with HIV by 80% by 2025 (compared to a 2010 baseline).
-

HIV testing

6. Which of the following HIV testing approaches are used in your country (select all that apply)?

- ☒ Client-initiated testing and counselling
☒ Provider-initiated testing and counselling
☒ Indicator condition testing
☒ Routine antenatal testing
☒ Dual HIV/syphilis rapid diagnostic testing (such as for pregnant women, partners or any population)
☒ Community-based testing
☒ Lay provider testing
☒ Self-testing
☒ Network-based testing (including partner services, social network testing, or family testing)
☐ Other (please specify): _____
-

7. Has your country adapted the recommendations from the 2024 World Health Organization (WHO) Consolidated guidelines on differentiated HIV testing services in a national process on testing guidelines?

- ☒ Yes, fully
☐ Yes, partially
☐ No
☐ Don't know
-

8. Has your country included self-testing as a national policy (either within the national testing policy/plan or as a stand-alone self-testing policy)?

8.a HIV self-testing

☒ Yes

☐ No

8.a.i If yes, is the HIV self-testing policy routinely implemented in your country?

☒ HIV self-testing policy is routinely implemented on a national scale

☐ HIV self-testing policy is routinely implemented on a subnational scale or in selected districts

☐ No, not implemented anywhere

8.a.ii If yes to Q8.a, has your country included HIV self-testing to support PrEP initiation or continuation (either within the national HIV testing policy or plan or as a standalone HIV self-testing policy)?

☒ Yes

☐ No

8.a.iii If yes to Q8.a.ii, is HIV self-testing to support PrEP routinely implemented in your country?

☒ Yes, routinely implemented on a national scale

☐ Yes, routinely implemented on a subnational scale or in selected districts

☐ No, only in pilot projects

☐ No, not implemented anywhere

8.a.iv If yes to Q8.a, has your country included HIV self-testing to support PEP initiation or continuation (either within the national HIV testing policy or plan or as a standalone HIV self-testing policy)?

☐ Yes

☒ No

8.a.v If yes to Q8.a.iv, is HIV self-testing to support PEP routinely implemented in your country?

☐ Yes, routinely implemented on a national scale

☐ Yes, routinely implemented on a subnational scale or in selected districts

☐ No, only in pilot projects

☐ No, not implemented anywhere

8.b Syphilis self-testing

☐ Yes

☒ No

8.b.i If yes, is the syphilis self-testing policy routinely implemented in your country?

☐ Syphilis self-testing policy is routinely implemented on a national scale

☐ Syphilis self-testing policy is routinely implemented on a subnational scale or in selected districts

☐ No, not implemented anywhere

8.c Hepatitis C self-testing

☐ Yes

☒ No

8.c.i If yes, is hepatitis C self-testing policy routinely implemented in your country?

☐ Hepatitis C self-testing policy is routinely implemented on a national scale

☐ Hepatitis C self-testing policy is routinely implemented on a subnational scale or in selected districts

☐ No, not implemented anywhere

8.d Other self-testing (please specify)

☐ Yes

☒ No

8.d.i If yes, is the other self-testing policy routinely implemented in your country?

☐ Other self-testing policy is routinely implemented on a national scale

☐ Other self-testing policy is routinely implemented on a subnational scale or in selected districts

☐ No, not implemented anywhere

9. Has your country included any form of network-based testing (partner services, social network testing, family testing) in a national policy (select all that apply)?

☒ Yes, partner services

☒ Yes, social network testing

☐ Yes, family testing

☐ No, none of the above

9.1 If yes, which of the following network-based testing approaches are being used (select all that apply)?

☒ Provider-assisted partner services

☐ Passive partner services

☒ Social network testing in key populations

☐ Social network testing in the general population

☐ Secondary distribution of HIV self-test kits to partners or network contacts

☒ Testing of biological children of people living with HIV

10. Has your country adopted or included dual HIV/syphilis rapid diagnostic tests for pregnant women and/or key populations as a national policy or plan?

☐ Yes, for pregnant women only

☐ Yes, for key populations only

☒ Yes, for both pregnant women and key populations

☐ No

10.1 If yes, is dual HIV/syphilis rapid diagnostic testing routinely implemented in your country?

10.1.a For pregnant women:

☒ Yes, routinely implemented on a national scale

☐ Yes, routinely implemented on a subnational scale or in select districts

☐ No, only in pilot projects

☐ No, not implemented anywhere

10.1.b For people from key populations:

☒ Yes, routinely implemented on a national scale, including subnational

☐ Yes, routinely implemented on a subnational scale or in selected districts

☐ No, only in pilot projects

☐ No, not implemented anywhere

11. Does your country use three consecutive reactive tests (3-test strategy/algorithm) for an HIV-positive diagnosis?

☐ Yes

☒ No

11.1 If yes to Q11, is the 3-test strategy routinely implemented in your country?

☐ Yes

☐ No

11.2 If no to Q11, does your country have a plan to adopt a 3-test strategy/algorithm for an HIV-positive diagnosis?

☐ Yes

☒ No

11.3 If yes to Q11.2, please indicate the year in which a national policy on 3-test strategy for an HIV-positive diagnosis is planned to be adopted.

☐ No planned year

☐ 2025

☐ 2026

☐ 2027

☐ 2028

11.4 Does your country use the following assays in the standard/routine national testing strategy/algorithm (select all that apply)?

☒ Rapid diagnostic (antibody test)

☒ Rapid diagnostic (antibody/antigen test)

☒ Immunoassay

☐ Western blotting

☒ Nucleic acid test

☐ Recency test

Antiretroviral therapy

12. Has your country adopted the recommendations on rapid Initiation of antiretroviral therapy in the 2021 World Health Organization (WHO) *Consolidated guidelines on HIV prevention, testing, treatment, service delivery and monitoring: recommendations for a public health approach*?

- ☒ Yes, rapid initiation within 7 days of HIV diagnosis
☐ No
☐ Other (please specify):

12.1 If your country has adopted a policy on rapid initiation of antiretroviral therapy, what is the status of implementation?

- ☐ Implemented in few (<50%) treatment sites
☐ Implemented in many (50–95%) treatment sites
☒ Implemented countrywide (>95% of treatment sites)
☐ Not implemented in practice
☐ Other (please specify): _____

13. Does your country have a policy to offer starting antiretroviral therapy on the same day as an HIV diagnosis, as part of the policy on rapid initiation of antiretroviral therapy?

- ☒ Yes
☐ No

13.1 If yes, what is the status of implementation?

- ☐ Implemented in few (<50%) treatment sites
☐ Implemented in many (50–95%) treatment sites
☒ Implemented countrywide (>95% of treatment sites)
☐ Not implemented in practice
☐ Other (please specify): _____

14. Is CD4 testing for diagnosing advanced HIV disease available?

- ☒ Yes
☐ No

14.1 Is yes, where is it available (select all that apply)?

- ☒ Point-of-care
☒ Facility laboratory
☒ Centralized laboratory
☐ Other (please specify): _____

14.2 If yes, in what percentage of sites (estimated) do clients have access to CD4 testing and return of results?

- ☐ In few (<50%) sites
☐ In many (50–95%) sites
☒ Countrywide (>95% of sites)
☐ Not implemented in practice
☐ Other (please specify): _____

14.3 If yes, what is the median time (in number of days) for the person to receive the CD4 result?

- ☒ Please specify: **1 week**
☐ Not available

15. Is nurse-initiated antiretroviral therapy allowed in your country for any of the following populations (select all that apply)?

- ☐ Adults, except pregnant women
☐ Pregnant women
☐ Adolescents (aged 10–19 years)
☐ Children younger than 10 years
☒ None of the above
-

16. Does your country have a national policy promoting community delivery of antiretroviral therapy (such as outside of health facilities)?

☒ Yes

☐ No

16.1 If yes, where is delivery in a community setting implemented?

- ☒ Nationally
- ☐ Regionally
- ☐ At pilot sites
- ☐ Other (please specify): _____

16.2 If yes, to which populations is antiretroviral therapy provided in community settings in your country (such as outside of health facilities)?

- ☒ For all people on antiretroviral therapy, including pregnant and breastfeeding women and children
- ☐ For all people on antiretroviral therapy, excluding pregnant and breastfeeding women and children
- ☐ For all people on antiretroviral therapy, including pregnant and breastfeeding women, but excluding children
- ☐ For all people on antiretroviral therapy, including children, but excluding pregnant and breastfeeding women
- ☐ For all people who are stable on antiretroviral therapy, according to the national guidelines
- ☐ Other (please specify): _____

16.3 If yes, which differentiated service delivery models is your country using for the pick-up of antiretroviral medicine (select all that apply)?

- ☐ Group models managed by health-care workers (such as adherence clubs, teen clubs)
- ☐ Group models managed by clients (such as community adherence groups, client-led ART delivery)
- ☒ Individual models based at facilities (such as multi-month ARV refills, fast track ARV pick-up)
- ☐ Individual models not based at facilities (such as community drug distribution points, ARV lockers, home ARV delivery, mobile clinics)
- ☐ Other (please specify): _____

17. Does your country have a national policy on the frequency of clinic visits for adults who are established* on antiretroviral therapy?

- ☒ Yes
- ☐ No

17.1 If yes, please specify the frequency of clinic visits in the national policy.

- ☐ Once a month
- ☐ Every 2 months
- ☐ Every 3 months
- ☒ Every 6 months
- ☐ Every 12 months

17.2 If yes, what is the status of implementation?

- ☐ Implemented in few (<50%) treatment sites
- ☐ Implemented in many (50–95%) treatment sites
- ☒ Implemented countrywide (>95% of treatment sites)
- ☐ Not implemented in practice
- ☐ Other (please specify): _____
-

18. Does your country have a national policy on how frequently adults who are established* on antiretroviral therapy should pick-up antiretroviral medicine?

☒ Yes

☐ No

18.1 If yes, please specify the frequency of antiretroviral medicine pick-up included in the national policy.

☐ Once a month

☐ Every 2 months

☒ Every 3 months

☐ Every 6 months

☐ Every 12 months

☐ Other (please specify): _____

18.2 If yes, what is the status of implementation?

☐ Implemented in few (<50%) treatment sites

☐ Implemented in many (50–95%) treatment sites

☒ Implemented countrywide (>95% of treatment sites)

☐ Not implemented in practice

☐ Other (please specify): _____

19. Do the country's national criteria for (or definition of) people established* on antiretroviral therapy include the following elements defined in the 2021 World Health Organization (WHO) *Consolidated guidelines on HIV prevention, testing, treatment, service delivery and monitoring: recommendations for a public health approach* (select all that apply)?

☒ Receiving antiretroviral therapy for at least 6 months

☒ No current illness (does not include well-controlled chronic health conditions)

☒ Good understanding of lifelong adherence

☒ Evidence of treatment success (i.e. at least one suppressed viral load result within the past six months)

☒ Other (please specify): **2 consecutive suppressed viral load results six months apart**

20. Does the country provide psychological support for adolescents living with HIV?

☒ Yes

☐ No

21. Does your country implement interventions to trace people who have disengaged from care and provide support for reengagement?

☒ Yes

☐ No

22. Please provide the country's national criteria for (or definition of) lost to follow-up. For guidance, the World Health Organization (WHO) defines "lost to follow-up" as a patient who has not received antiretroviral medicines within 28 days of their last missed drug collection appointment.¹

90 days since their last missed appointment

23. Has your country adopted the recommendations in the 2021 World Health Organization (WHO) *Consolidated guidelines on HIV prevention, testing, treatment, service delivery and monitoring: recommendations for a public health approach* to offer a package of interventions to all patients presenting with advanced HIV disease (defined by WHO as CD4<200)?

☒ Yes, fully adopted

☐ Yes, partially adopted (only for specific interventions and/or populations, such as children, adolescents or adults) (please specify): _____

☐ No

23.1 If yes, how widely is it implemented?

☐ Implemented in few (<50%) treatment sites

☐ Implemented in many (50–95%) treatment sites

☒ Implemented countrywide (>95% of treatment sites)

☐ Not implemented in practice

☐ Other (please specify): _____

¹ Source: Consolidated guidelines on person-centred HIV strategic information: strengthening routine data for impact. Geneva: World Health Organization; 2022, page 127)

24. Which of the following components of the package of advanced HIV disease interventions for tuberculosis (TB), severe bacterial infections and cryptococcal meningitis recommended in the 2021 WHO *Consolidated guidelines on HIV prevention, testing, treatment, service delivery and monitoring: recommendations for a public health approach* are included in the national policy on antiretroviral therapy for adults, adolescents and children (select all that apply)?

- ☒ Baseline CD4 count test to diagnose advanced HIV disease
- ☒ Molecular diagnostic tests for TB diagnosis
- ☐ Urine LF-LAM for TB diagnosis
- ☐ Cryptococcal antigen (CrAg) screening
- ☒ Co-trimoxazole prophylaxis
- ☐ TB preventive treatment
- ☒ Fluconazole empirical prophylaxis
- ☐ Fluconazole pre-emptive therapy
- ☒ Rapid antiretroviral therapy initiation
- ☒ Adapted adherence support
- ☐ Other (please specify): _____

25. Which of the following service provision modalities are included in the national policy on antiretroviral therapy for adults, adolescents and children (select all that apply)?

- ☒ Tuberculosis (TB) service providers provide antiretroviral therapy in TB clinics for the duration of TB treatment
 - ☐ Antiretroviral therapy providers provide TB treatment in antiretroviral therapy settings for the duration of TB treatment
 - ☒ Maternal, newborn and child health (MNCH) service providers provide antiretroviral therapy in MNCH clinics
 - ☒ Antiretroviral therapy providers deliver antiretroviral therapy for pregnant women
 - ☒ Antiretroviral therapy providers deliver antiretroviral therapy for newborns, infants and children
 - ☒ Nutrition assessment, counselling and support provided to malnourished people living with HIV
 - ☐ Antiretroviral therapy delivered in settings providing opioid agonist maintenance therapy
 - ☒ Primary health-care providers deliver antiretroviral therapy in primary health care for adults and adolescents
 - ☒ Primary health-care providers deliver antiretroviral therapy in primary health-care settings for children
 - ☒ Psychosocial support strategies for patient-centered care (e.g. support groups, enhanced adherence counselling, support for disclosure or referral for psychological/socioeconomic services) linked to facilities
 - ☒ Patient-centered support (e.g. counselling, enhanced adherence counselling, support for disclosure or referral for psychological/socioeconomic services) separated from facilities
 - ☒ Key population-friendly services
 - ☐ Adolescent-friendly health services
 - ☒ Antiretroviral therapy delivered in the community as part of a differentiated care model
 - ☒ Antiretroviral therapy providers carry out cardiovascular disease screening and management
 - ☒ Antiretroviral therapy providers carry out mental health screening and treatment
 - ☐ Other (please specify): _____
-

26. Do patients pay any routine user fees or charges for services when visiting a public sector health facility?

- ☐ Yes
☒ No

26.1 If yes, is there a specific formal fee or an informal/variable fee for the following?

26.1.a. HIV testing

- ☐ Formal
☐ Informal

26.1.b. Dispensing of pre-exposure prophylaxis (PrEP)

- ☐ Formal
☐ Informal

26.1.c. Primary care appointment

- ☐ Formal
☐ Informal

26.1.d. Patient cards

- ☐ Formal
☐ Informal

26.1.e. Diagnostic services (including viral load test)

- ☐ Formal
☐ Informal

26.1.f. Dispensing of HIV treatment

- ☐ Formal
☐ Informal

26.1.g. Dispensing of prevention and treatment of co-infections

- ☐ Formal
☐ Informal

26.1.h. Dispensing of other co-therapies (eg: medicines for NCDs, sexual & reproductive health, immunizations)

- ☐ Formal
☐ Informal
-

Antiretroviral therapy regimens

Adults and adolescents

27. Based on the recommendations in the 2021 WHO *Consolidated guidelines on HIV prevention, testing, treatment, service delivery and monitoring: recommendations for a public health approach*, is TDF + 3TC (or FTC) + DTG the preferred first-line antiretroviral combination for treatment initiation in national guidelines for the following:
- 27.a Adults and adolescents?
- ☒ Yes
- ☐ No, TDF + 3TC (or FTC) + DTG is included as alternative first-line regimen
- ☐ No
- 27.a.i. If yes, what is the status of implementation?
- ☐ Implemented in few (<50%) treatment sites
- ☐ Implemented in many (50–95%) treatment sites
- ☒ Implemented countrywide (>95% of treatment sites)
- ☐ Not implemented in practice
- Other (please specify): _____
- 27.a.ii If no, what is (are) the preferred option(s):
- ☐ TDF + 3TC (or FTC) + EFV 600 mg
- ☐ TDF + 3TC + EFV 400 mg
- ☐ ABC + 3TC + DTG
- ☐ TAF + 3TC (or FTC) + DTG
- ☐ Other regimens (please specify): _____
- 27.a.iii If no, is there a plan to adopt TDF + 3TC (or FTC) + DTG as the preferred first-line antiretroviral combination for treatment initiation in 2025?
- ☐ Yes
- ☐ No
- 27.b Women of childbearing age?
- ☒ Yes
- ☐ No
- 27.b.i If no, what is (are) the preferred option(s):
- ☐ TDF + 3TC (or FTC) + EFV 600 mg
- ☐ TDF + 3TC + EFV 400 mg
- ☐ TAF + 3TC (or FTC) + DTG
- ☐ Other regimens (please specify): _____
- 27.b.ii If no, is there a plan to adopt TDF + 3TC or (FTC) + DTG as the preferred first-line antiretroviral medicine combination for treatment initiation in 2025?
- ☐ Yes
- ☐ No
- 27.c. Pregnant and/or breastfeeding women?
- ☒ Yes
- ☐ No
- 27.c.i If no, what is (are) the preferred option(s):
- ☐ TDF + 3TC (or FTC) + EFV 600 mg
- ☐ TDF + 3TC + EFV 400 mg
- ☐ TAF + 3TC (or FTC) + DTG
- ☐ Other regimens (please specify): _____
- 27.c.ii If no, is there a plan to adopt TDF + 3TC or (FTC) + DTG as the preferred first-line antiretroviral medicine combination for treatment initiation in 2025?
- ☐ Yes
- ☐ No
-

28. **Does your country use fixed-dose combination (FDC) antiretroviral therapy as the preferred first-line therapy (select all that apply)?**

☒ Yes, 3-drug fixed-dose combination, taken once a day

☐ Yes, 2-drug fixed-dose combination plus 1 other drug, taken once a day

☐ No

☐ Other (please specify): _____

29. **Is a DTG-based regimen included in the national guidelines as an option for second-line antiretroviral combination for adults and adolescents with HIV?**

☐ Yes, as preferred option

☐ Yes, as alternative option

☐ No

☒ Other (please specify): **Adolescents second-line option**

30. **Is atazanavir/ritonavir (ATV/r) included in national guidelines as a protease inhibitor option for second-line antiretroviral combination for adults and adolescents with HIV?**

☒ Yes, as preferred option

☐ Yes, as alternative option

☐ Yes, as third-line option

☐ No

☐ Other (please specify): _____

31. **Is lopinavir/ritonavir (LPV/r) included in national guidelines as a protease inhibitor option for second-line antiretroviral combination for adults and adolescents with HIV?**

☒ Yes, as preferred option

☐ Yes, as alternative option

☐ Yes, as third-line option

☐ No

☐ Other (please specify): _____

32. **Is darunavir/ritonavir (DRV/r) included in national guidelines as a protease inhibitor option for second-line antiretroviral combination for adults and adolescents with HIV?**

☐ Yes, as preferred option

☐ Yes, as alternative option

☒ Yes, as third-line option

☐ No

☐ Other (please specify): _____

Children

- 33. Are DTG regimens the preferred treatment initiation option in the national guidelines for all infants and children with HIV?**
- ☒ Yes, for all children older than 4 weeks and weighing more than 3kg
- ☐ Yes, but only for children weighing more than 20kg
- ☐ No
- ☐ Other (please specify): _____
- 33.1 If DTG is not the preferred treatment option for infants and children older than 4 weeks and weighing more than 3kg, are LPV/r-based regimens the preferred treatment option?**
- ☐ Yes, for all
- ☐ No, but only for children weighing less than 20kg
- ☐ No
-
- 34. What is the recommended NRTI backbone in the national guidelines for treatment initiation in children?**
- ☐ TDF + 3TC (or FTC)
- ☐ AZT + 3TC (or FTC)
- ☒ ABC + 3TC (or FTC)
- ☐ Other (please specify): _____
-
- 35. Is DTG recommended as the preferred second-line option for children failing NNRTI*-based regimens?**
- ☐ Yes, for all children older than 4 weeks
- ☒ Yes, for children weighing more than 20 kg
- ☐ No
- ☐ Other (please specify): _____
-
- 36. Is DTG recommended as the preferred second-line option for children failing protease inhibiting-based regimens?**
- ☒ Yes, for all children older than 4 weeks and weighing more than 3kg
- ☐ Yes, but only for children weighing more than 20kg
- ☐ No
- ☐ Other (please specify): _____
-
- 37. What is the recommended second-line option for children failing DTG-based regimens?**
- ☒ LPV/r
- ☐ ATV/r
- ☐ Other (please specify): _____
-
- 38. Is a darunavir/ritonavir (DRV/r)-based regimen included in the national guidelines as an option for antiretroviral therapy of children with HIV?**
- ☐ Yes, as preferred second-line option
- ☐ Yes, as alternative second-line option
- ☐ Yes, only as third-line option
- ☒ Not recommended
- ☐ Other (please specify): _____
-
- 39. Are any of the following early childhood development activities integrated into HIV programmes (select all that apply)?**
- ☐ Responsive caregiving
- ☐ Promote early learning
- ☒ Integrate caregiving and nutrition interventions
- ☒ Support maternal mental health
- ☐ None of the above
-

Viral load

40. Please identify the measured threshold in national treatment guidelines at which viral load suppression² in an individual is defined as suppressed:

- ☒ ≤1000 copies/ml
☐ ≤400 copies/ml
☐ ≤200 copies/ml
☐ ≤50 copies/ml
☐ Not detected by assay or sample type used
☐ Other (please specify): _____
-

41. Does your country have a current national policy on routine viral load testing* for monitoring antiretroviral therapy?

41.a Adults and adolescents

- ☒ Yes
☐ No

41.a.i If yes, what is the status of implementation?

- ☐ Implemented in few (<50%) treatment sites
☐ Implemented in many (50–95%) treatment sites
☒ Implemented countrywide (>95% of treatment sites)
☐ Not implemented in practice
☐ Other (please specify): _____

41.b. For children

- ☒ Yes
☐ No

41.b.i If yes, what is the status of implementation?

- ☐ Implemented in few (<50%) treatment sites
☐ Implemented in many (50–95%) treatment sites
☒ Implemented countrywide (>95% of treatment sites)
☐ Not implemented in practice
☐ Other (please specify): _____
-

42. Does your country have a current national policy on point-of-care viral load testing?

- ☐ Yes
☒ No

42.1 If yes, what is the status of implementation?

- ☐ Implemented in few (<50%) treatment sites
☐ Implemented in many (50–95%) treatment sites
☐ Implemented countrywide (>95% of treatment sites)
☐ Not implemented in practice
☐ Other (please specify): _____
-

43. Are dried blood spot specimens recommended in the national policy for viral load testing?

- ☐ Yes
☒ No
☐ Other (please specify): _____

43.1 If yes, what is the level of implementation?

- ☐ Fully
☐ Partially
☐ Not implemented
-

² The viral suppression threshold for may differ from the threshold to determine treatment failure.

HIV drug resistance and toxicity monitoring

44. ☒ Does your country have a national plan to monitor HIV drug resistance?

☐ Yes

☐ No

44.1 If yes, please specify the years covered by the plan: **2022 - 2025**

45. In the past three years, has your country carried out HIV drug resistance (HIVDR) surveillance according to any of the following World Health Organization (WHO) protocols?

45.a Pre-treatment drug resistance (PDR) surveys³

☐ Yes

☐ No, but there is a plan to implement the PDR survey this year

☒ No, and there is no plan to implement the PDR survey this year

45.a.i. If yes, please specify the year the last PDR survey started: _____

45.b HIV drug resistance survey among individuals exposed to pre-exposure prophylaxis (PrEP) diagnosed with HIV infection

☐ Yes

☐ No, but there is a plan to implement the survey this year

☒ No, and there is no plan to implement the survey this year

45.b.i. If yes, please specify the year the last survey started: _____

45.c Acquired drug resistance surveys among adults⁴

☐ Yes

☐ No, but there is a plan to implement the survey this year

☒ No, and there is no plan to implement the survey this year

45.c.i. If yes, please specify the year the last survey started: _____

45.d Acquired drug resistance surveys among children

☐ Yes

☐ No, but there is a plan to implement the survey this year

☒ No, and there is no plan to implement the survey this year

45.d.i. If yes, please specify the year the last survey started: _____

45.e HIV drug resistance among infants (<18 months) using early infant diagnosis⁵

☐ Yes

☐ No, but there is a plan to implement the infant survey this year

☒ No, and there is no plan to implement the infant survey this year

45.e.i. If yes, please specify the year the last infant survey started: _____

45.f ☒ Survey or routine monitoring of clinic performance using early warning indicators (EWI) for HIV drug resistance

☐ Yes

☐ No

45.f.i. If yes, please specify:

☐ Year it was last monitored: **2024**

☐ Number of clinics monitored: **56**

45.g The early warning indicators (EWI) for HIV drug resistance were collected through:

☐ EWI survey in a sample of clinics

☒ Routine patient monitoring systems

³ For more details, please see: Surveillance of HIV drug resistance in adults initiating antiretroviral therapy. Geneva: WHO; 2014 (<https://www.who.int/publications/i/item/9789241507196>).

⁴ For more details, please see: Surveillance of HIV drug resistance in adults receiving ART. Geneva: WHO; 2014 (<https://www.who.int/publications/i/item/9789241507073>).

⁵ For more details, please see: HIV drug resistance. In: World Health Organization: Global HIV Programme: Treatment & Care [website]. Geneva: WHO; c2018 (<https://www.who.int/teams/global-hiv-hepatitis-and-stis-programmes/hiv/treatment/hiv-drug-resistance>).

46. Does your country have a national policy for HIV drug resistance testing for individual patients who fail antiretroviral therapy for clinical management?

☒ Yes

☐ No

46.1 If yes, which of the following groups are considered in the policy (select all that apply)?

☐ Adults failing DTG-based first-line antiretroviral therapy

☐ Children failing DTG-based first-line antiretroviral therapy

☒ Adults failing DTG-based second-line antiretroviral therapy

☒ Children failing DTG-based second-line antiretroviral therapy

☒ Adults failing protease inhibitor-based second-line antiretroviral therapy

☒ Children failing protease inhibitor-based second-line antiretroviral therapy

☒ Patients failing third-line antiretroviral therapy

☐ Other (please specify): _____

47. Does your country have a national policy for HIV drug resistance testing for individual patients who initiate antiretroviral therapy?

☐ Yes

☒ No

47.1 If yes, which of the following groups are considered in the policy (select all that apply)?

☐ All adults initiating first-line DTG-based first-line antiretroviral therapy

☐ People with a history of tenofovir-based PrEP for the prevention of HIV

☐ People with a prior history of cabotegravir-based PrEP for the prevention of HIV

☐ Infants and children initiating first-line antiretroviral therapy

☐ Pregnant women initiating first-line antiretroviral therapy

☐ Other (please specify): _____

48. Excluding passive pharmacovigilance approaches, does your country make an ongoing systematic effort to monitor the toxicity of antiretroviral medicines in the country?

☐ Yes

☒ No

48.1 If yes, what approaches are used (select all that apply)?

☐ Routine toxicity monitoring as part of the national monitoring and evaluation system

☐ Active toxicity monitoring/surveillance within cohorts in adults

☐ Active toxicity monitoring/surveillance within cohorts in adolescents and children

☐ Pregnancy registry and surveillance of birth defects

49. Have toxicity monitoring approaches been introduced to monitor adverse drug reactions to DTG use?

☒ Yes

☐ No

49.1 If yes, what approaches are used (select all that apply)?

☒ Routine toxicity monitoring as part of the national monitoring and evaluation system

☐ Active toxicity monitoring/surveillance within cohorts in adults

☐ Active toxicity monitoring/surveillance within cohorts in adolescents and children

☐ Pregnancy registry and surveillance of birth defects

49.2 If yes, has training of health-care workers on the management, capture and reporting of adverse drug reactions related to DTG been implemented?

☒ Yes

☐ No

Adherence and retention

50. Does your country have national policies and/or strategies on adherence support (community and facility-based)?

☒ Yes

☐ No

51. Are any of the following adherence support services being implemented in your country (select all that apply)?

☒ Peer counsellors

☒ Text messages

☒ Use of reminder devices

☒ Patient reintegration follow-up calls/home visits

☒ Enhanced adherence counselling

☒ Referral to psychological/socioeconomic support

☒ Cognitive behavioural therapy

☒ Behavioural skills training/medication adherence training

☒ Fixed-dose combinations and once-daily regimens

☒ Case management

☒ Peer navigation

☐ Other (please specify): _____

52. Does your country have national policies and/or strategies on retention in antiretroviral therapy?

☒ Yes

☐ No

53. Are any of the following retention support services being implemented in your country (select all that apply)?

☒ Community-based interventions

☒ Adherence clubs and peer support

☐ Other (please specify): _____

54. Are treatment literacy programmes available in your country to people living with HIV, including information on side effects, drug resistance, etc.?

☒ Yes

☐ No

Tuberculosis/HIV

55. Are the following screening tools recommended for people living with HIV in national guidelines related to tuberculosis (TB) and/or HIV (select all that apply)?

- ☒ World Health Organization-recommended four symptom screen for adults and adolescents (>10 years)
- ☐ C-reactive protein (CRP) for adults and adolescents (>10 years)
- ☐ Chest X-ray for adults and adolescents (>10 years)
- ☐ Molecular World Health Organization-approved rapid diagnostic tests for TB (mWRD) for adults and adolescents (>10 years)
- ☒ Symptom screen, including cough, fever, poor weight gain or close contact with a TB patient for children <10 years
- ☐ None of the above

56. Has your country adopted the 2019 WHO policy update on the use of lateral flow urine lipoarabinomannan assay (LF-LAM) for the diagnosis and screening of active tuberculosis in people living with HIV (select all age groups that apply)?

- ☐ Yes, for adults and adolescents (>10 years)
- ☐ Yes, for children (<10 years)
- ☒ No

57. Which of the following regimens are recommended for tuberculosis (TB) preventive treatment in national guidelines?

57.a Adults and adolescents living with HIV (select all that apply)

- ☐ 6 months of daily isoniazid monotherapy (6H)
- ☐ 9 months of daily isoniazid monotherapy (9H)
- ☐ 4 months of daily rifampicin (4R)
- ☐ 3 months of weekly rifapentine plus isoniazid (3HP)
- ☐ 3 months of daily rifampicin plus isoniazid (3HR)
- ☐ 1 month of daily rifapentine plus isoniazid (1HP)
- ☐ 6 months of daily levofloxacin monotherapy (6Lfx)
- ☐ Other (please specify): _____
- ☒ None of the World Health Organization recommended TB preventive treatment regimens are recommended in national guidelines for adults and adolescents

57.a.i If more than one regimen is recommended, which is the preferred regimen?

- ☐ 6 months of daily isoniazid monotherapy (6H)
- ☐ 9 months of daily isoniazid monotherapy (9H)
- ☐ 4 months of daily rifampicin (4R)
- ☐ 3 months of weekly rifapentine plus isoniazid (3HP)
- ☐ 3 months of daily rifampicin plus isoniazid (3HR)
- ☐ 1 month of daily rifapentine plus isoniazid (1HP)
- ☐ 6 months of daily levofloxacin monotherapy (6Lfx)
- ☐ Other (please specify): _____

57.b Children living with HIV (select all that apply)

- ☐ 6 months of daily isoniazid monotherapy (6H)
- ☐ 9 months of daily isoniazid monotherapy (9H)
- ☐ 4 months of daily rifampicin (4R)
- ☐ 3 months of weekly rifapentine plus isoniazid (3HP)
- ☐ 3 months of daily rifampicin plus isoniazid (3HR)
- ☐ 6 months of daily levofloxacin monotherapy (6Lfx)
- ☐ Other (please specify): _____
- ☒ TB preventive treatment not recommended in national guidelines for children

57.b.i If more than one regimen is recommended, which is the preferred regimen?

- ☐ 6 months of daily isoniazid monotherapy (6H)
 - ☐ 9 months of daily isoniazid monotherapy (9H)
 - ☐ 4 months of daily rifampicin (4R)
 - ☐ 3 months of weekly rifapentine plus isoniazid (3HP)
 - ☐ 3 months of daily rifampicin plus isoniazid (3HR)
 - ☐ 6 months of daily levofloxacin monotherapy (6Lfx)
 - ☐ Other (please specify): _____
-

58. Are the following required in national guidelines prior to initiating tuberculosis (TB) preventive treatment in people with HIV?

58.a Skin tests or interferon-gamma release assay (IGRA)

☐ Yes, for all

☐ No

☐ Only if available

N/A TB preventive therapy not in national guidelines

58.b X-ray

☐ Yes, for all

☐ No

☐ Only if available

59. In the last reporting period, has there been a stock-out* of any of the following?

59.a Isoniazid

☐ Yes, at the national level

☐ Yes, at the local level

☒ No

59.b Vitamin B6

☐ **N/A** Yes, at the national level

☐ Yes, at the local level

☐ No

59.c Rifapentine (including fixed-dose combinations with isoniazid)

☐ Yes, at the national level

☐ Yes, at the local level

☒ No

60. What is the status of integration of the following HIV and tuberculosis (TB) services?

60.a World Health Organization-recommended rapid molecular diagnostics (e.g. Xpert MTB/RIF) are co-located:

☐ In few (<50%) health facilities providing HIV testing and care

☐ In many (50–95%) health facilities providing HIV testing and care

☐ Countrywide (>95% of health facilities providing HIV testing and care)

☐ Not integrated in practice

☒ Other (please specify): **Sample collection is co-located but rapid molecular diagnostics for TB is conducted at centralized laboratory**

60.b People living with HIV who have tuberculosis (TB) received antiretroviral medicines at the same place as they receive their TB treatment:

☒ In few (<50%) health facilities

☐ In many (50–95%) health facilities

☐ Countrywide (>95% of health facilities)

☐ Not integrated in practice

☐ Other (please specify): _____

60.c Antiretroviral therapy is initiated by the same health-care worker providing tuberculosis (TB) treatment for people living with HIV who have TB:

☐ In few (<50%) health facilities

☐ In many (50–95%) health facilities

☒ countrywide (>95% of health facilities)

☐ Not integrated in practice

☐ Other (please specify): _____

60.d Antiretroviral therapy and tuberculosis (TB) treatment for people living with HIV who have TB are monitored by one health-care worker:

☐ In few (<50%) health facilities

☐ In many (50–95%) health facilities

☐ Countrywide (>95%) of health facilities

☐ Not integrated in practice

☒ Other (please specify): **initially for the intensive phase but during the continuation phase there is integration within the community**

3. End paediatric AIDS and eliminate vertical transmission

- Ensure that 75% of all children living with HIV have suppressed viral loads by 2023 and 86% by 2025, in line with the 95–95–95 HIV treatment targets.
- Ensure that 95% of pregnant women have access to testing for HIV, syphilis, hepatitis B and other sexually transmitted infections by 2025.
- Ensure that 95% of pregnant and breastfeeding women in high HIV burden settings have access to retesting during late pregnancy and in the post-partum period by 2025.
- Ensure that all pregnant and breastfeeding women living with HIV are receiving life-long antiretroviral therapy, with 95% achieving and sustaining viral suppression before delivery and during breastfeeding by 2025.
- Ensure that all HIV-negative pregnant and breastfeeding women in high HIV burden settings—or those who have male partners at high risk of HIV in all settings—have access to combination prevention, including pre-exposure prophylaxis (PREP), and that 90% of their male partners who are living with HIV are continuously receiving antiretroviral therapy.
- Ensure that 95% of children with perinatal HIV exposure are tested by two months of age and after the cessation of breastfeeding.

Prevention of vertical transmission of HIV

61. Does your country have a policy on retesting HIV-negative women during pregnancy, delivery and/or the post-partum/breastfeeding period?



Yes



No

61.1 If yes, when is retesting done?

61.1.a

During pregnancy



Yes



No

61.1.a.i If yes, which month of pregnancy: **Third Trimester**

61.1.b At delivery



Yes



No

61.1.c Post-partum/breastfeeding



Yes



No

61.1.c.i If yes, how long after delivery (in months): _____

62. Does your country have a national plan for the elimination of vertical transmission of HIV:



Yes



No

62.1 If yes, please specify:

62.1.a Target(s) for the vertical transmission rate (%): **< 2%**

62.1.b Year: **2024**

62.1.c Elimination target(s) (such as the number of cases/100 000 population): **< 0.3/1000 live births**

62.1.d Year: **2024**

63. Is your country implementing a treat all policy for pregnant and breastfeeding women living with HIV?



Yes



No

64. What is the current nationally recommended regimen for infants with perinatal HIV exposure for preventing vertical transmission of HIV?

☐ Please specify the infant prophylaxis regimen: **Nevirapine 2 mg/kg immediately and Zidovudine 4 mg/kg**

☐ Recommended duration of the regimen: **6 weeks**

64.a Are different regimens recommended for high-risk infants?

☒ Yes

☐ No

64.a.i If yes, please specify the regimens: **Nevirapine for 6 weeks and Zidovudine for 6 weeks**

64.a.ii What is the definition of "high-risk infant" in the national policy (select all that apply)?

☒ Born to women with established HIV infection who have received less than 4 weeks of antiretroviral therapy at the time of delivery

☒ Born to women with established HIV infection with viral load >1000 copies/mL in the 4 weeks before delivery (if viral load is available)

☒ Born to women with incident HIV infection during pregnancy or breastfeeding

☐ Born to women identified for the first time during the postpartum period, with or without a negative HIV test prenatally

☐ Other (please specify): _____

65. Does your country have a national recommendation on infant and young child feeding for infants with perinatal HIV exposure?

☐ Yes, breastfeeding

☒ Yes, replacement feeding

☐ Yes, both are recommended, left to individual choice or different settings

☐ No

65.1 If breastfeeding is recommended for HIV-positive women and infants with perinatal HIV exposure, is the recommended duration specified?

☐ Yes (please specify the duration in months): _____

☐ No

66. Is food and nutrition support in your country integrated within prevention of vertical transmission programmes?

☐ Implemented in few (<50%) maternal and child health sites

☐ Implemented in many (50–95%) maternal and child health sites

☒ Implemented countrywide (>95% of maternal and child health sites)

☐ Not implemented in practice

☐ Other (please specify): _____

67. Does your country have a national strategy on interventions at delivery for women living with HIV who have not previously been tested for HIV?

☒ Yes, fully implemented

☐ Yes, partially implemented

☐ Yes, but not implemented

☐ No

68. Does your country have a policy on viral load testing for women during pregnancy, delivery and/or the post-partum/ breastfeeding period?

☒ Yes

☐ No

68.1 If yes, when is viral load testing done?

68.1.a During pregnancy

☒ Yes

☐ No

68.1.a.i If yes, which month of pregnancy: **Routinely in each trimester**

68.1.b At delivery

☐ Yes

☒ No

68.1.c Post-partum/breastfeeding

- ☐ Yes
☒ No

68.1.c.i If yes, how long after delivery (in months): _____

Elimination of vertical transmission of syphilis

69. Does your country have a national plan for the elimination of vertical transmission of syphilis?

- ☒ Yes, integrated with HIV or other elimination initiative(s)
☐ Yes, stand-alone (not integrated with HIV or other elimination initiatives)
☐ No national plan

69.1 If yes, when was the national plan last updated?

- ☐ 2020 or before
☒ 2021
☐ 2022
☐ 2023
☐ 2024
-

70. Does your country have a national policy for routinely screening pregnant women for syphilis?

- ☒ Yes
☐ No

70.1 If yes, what is the national testing algorithm for the routine screening of pregnant women for syphilis?

- ☐ Rapid treponemal test only (either syphilis only or dual HIV/syphilis test)
☐ Laboratory-based treponemal test only (e.g. Treponema pallidum haemagglutination assay [TPHA], T. pallidum particle agglutination assay [TPPA], or enzyme immunoassay)
☒ Non-treponemal test only (e.g. venereal disease research laboratory [VDRL] or rapid plasma reagin [RPR])
☒ Rapid treponemal test followed by non-treponemal test
☐ Laboratory-based treponemal test followed by non-treponemal test
☐ Non-treponemal test followed by treponemal test
☐ Other (please specify): _____
-

71. Does your country have national guidelines to treat pregnant women with syphilis?

- ☒ Yes
☐ No
-

72. Does your country have a national policy on the clinical follow-up of infants born to syphilis-positive mothers?

- ☒ Yes
☐ No
-

73. Is congenital syphilis a notifiable condition?

- ☒ Yes
☐ No
-

74. Does the national definition for congenital syphilis include stillbirths?

- ☒ Yes
☐ No
-

Prevention of vertical transmission of hepatitis B virus

75. Does your country have a national plan for the elimination of vertical transmission of hepatitis B virus (HBV)?

- ☐ Yes
☒ No

76. Does your country have a policy on testing women for HBV during pregnancy?

- ☐ Yes
☒ No

76.1 If yes, what is the status of implementation?

- ☐ Implemented in few (<50%) antenatal care clinics
☐ Implemented in many (50–95%) antenatal care clinics
☐ Implemented countrywide (>95% of antenatal care clinics)
☐ Not implemented in practice
☐ Other (please specify) _____:

76.2 If yes, do all HBsAg positive pregnant women have access to HBeAg or HBV DNA testing?

- ☐ Yes
☐ No

77. Does your country have a national recommendation on hepatitis B testing among infants exposed to HBV ?

- ☐ Yes
☒ No

77.1 If yes, please specify the age (in months) at which infants exposed to HBV are tested for HBV: _____

78. Does your country have a policy on universal timely hepatitis B birth dose vaccination to all newborns (provided within 24 hours of birth)?

- ☒ Yes
☐ No

78.1 If no, does your country have a policy on targeted timely hepatitis B birth dose vaccination to all newborns exposed to HBV (provided within 24 hours of birth)?

- ☐ Yes
☐ No

Infant diagnosis

79. Do your national guidelines recommend that infants with perinatal HIV exposure be tested for HIV as follows (select all that apply)?

- ☒ Nucleic acid testing at birth
☒ Nucleic acid testing at 6 weeks
☐ Nucleic acid testing at 9 months
☒ Antibody test at 18 months
☒ Antibody test after 3 months from cessation of breastfeeding
☒ Other (please specify): **Nucleic acid testing at 4 months**

80. In addition to prevention of vertical transmission settings, do any of the following sites in your country carry out HIV testing of children (select all that apply)?

- ☐ Paediatric inpatient wards
☐ Nutrition centres
☐ Immunization clinics
☐ Outpatient clinics
☐ Tuberculosis (TB) clinics
☒ Other (please specify): **Sample collection occurs at paediatric inpatient wards, nutrition centres and outpatient clinic but testing for children is centralised**
-

81. Does your country have a policy or recommendation for point-of-care infant diagnosis testing?

- ☐ Yes
☒ No

81.1 If yes, where is it implemented?

- ☐ Implemented in few (<50%) sites
☐ Implemented in many (50–95%) sites
☐ Implemented countrywide (>95% of sites)
☐ Not implemented in practice
☐ Other (please specify): _____

Child antiretroviral therapy

82. Does your country have a national policy on the frequency of clinic visits for children who are established* on antiretroviral therapy?

- ☒ Yes
☐ No

82.1 If yes, please specify the frequency of clinic visits in the national policy:

- ☐ Once a month
☐ Every 2 months
☒ Every 3 months
☐ Every 6 months
☐ Every 12 months
☐ Other (please specify): _____

82.2 If yes, what is the status of implementation?

- ☐ Implemented in few (<50%) treatment sites
☐ Implemented in many (50–95%) treatment sites
☒ Implemented countrywide (>95% of treatment sites)
☐ Not implemented in practice
☐ Other (please specify): _____

83. Does your country have a national policy on how frequently children who are established* on antiretroviral therapy should pick up antiretroviral medicine?

- ☒ Yes
☐ No

83.1 If yes, please specify the frequency of antiretroviral medicine pick-up included in the national policy:

- ☐ Once a month
☐ Every 2 months
☒ Every 3 months
☐ Every 6 months
☐ Every 12 months
☐ Other (please specify): _____

83.2 If yes, what is the status of implementation?

- ☐ Implemented in few (<50%) treatment sites
☐ Implemented in many (50–95%) treatment sites
☒ Implemented countrywide (>95% of treatment sites)
☐ Not implemented in practice
☐ Other (please specify): _____

84. Are cohorts of children receiving antiretroviral therapy monitored (i.e., ensuring that these children are alive and receiving antiretroviral therapy) in national registers at six-month and 12-month intervals?

- ☒ Yes
☐ No
-

85. **When is a child who initiated antiretroviral therapy considered lost to follow-up in your country?**

- ☐ Has not been seen for HIV care or pharmacy pick-up in 1 month
☐ Has not been seen for HIV care or pharmacy pick-up in 2 months
☒ Has not been seen for HIV care or pharmacy pick-up in 3 months
☐ Has not been seen for HIV care or pharmacy pick-up in 6 months
-

86. **Does your country have a strategy or plan to ensure that adolescents born with HIV are not lost to follow-up as they transition into adult HIV care?**

- ☒ Yes
☐ No
-

87. **Are growth monitoring and nutrition programmes for children integrated with HIV testing and treatment in your country?**

- ☐ Implemented in few (<50%) treatment sites
☐ Implemented in many (50–95%) treatment sites
☒ Implemented countrywide (>95% of treatment sites)
☐ Not implemented in practice
☐ Other (please specify): _____
-

4. Gender equality and empowerment of women and girls

- Reduce to no more than 10% the number of women, girls and people living with, at risk of and affected by HIV who experience gender-based inequalities and sexual and gender-based violence.
- Ensure that 95% of women and girls of reproductive age have their HIV and sexual and reproductive health-care service needs met, including antenatal and maternal care, information and counselling

Violence

88. Does your country have at least one service delivery point that provides elements of comprehensive post-rape care as per World Health Organization (WHO) guidelines? The elements are: (1) first-line support, psychological first aid and psychosocial support; (2) emergency contraception; (3) sexually transmitted infection prophylaxis or treatment; (4) HIV post-exposure prophylaxis (PEP); and (5) safe abortion to the full extent of the law.

- ☐ Yes, provides all 5 elements
- ☒ Yes, provides 4 out of 5 elements
- ☐ Yes, provides 1 to 3 elements
- ☐ No services delivery point provides any of these elements

88.1 If yes, what proportion of health facilities provides each of the following elements of comprehensive post-rape care as per World Health Organization (WHO) guidelines:

88.1.a First-line support, psychological first aid and psychosocial support

- ☐ <50% of health facilities
- ☐ 50–80% of health facilities
- ☒ >80% of health facilities
- ☐ Not provided in any health facility
- ☐ Don't know

88.1.b Emergency contraception

- ☐ <50% of health facilities
- ☐ 50–80% of health facilities
- ☒ >80% of health facilities
- ☐ Not provided in any health facility
- ☐ Don't know

88.1.c Sexually transmitted infection treatment or prophylaxis

- ☐ <50% of health facilities
- ☐ 50–80% of health facilities
- ☒ >80% of health facilities
- ☐ Not provided in any health facility
- ☐ Don't know

88.1.d HIV post-exposure prophylaxis (PEP)

- ☐ <50% of health facilities
- ☐ 50–80% of health facilities
- ☒ >80% of health facilities
- ☐ Not provided in any health facility
- ☐ Don't know

88.1.e Safe abortion to the full extent of the law

- ☐ <50% of health facilities
- ☐ 50–80% of health facilities
- ☐ >80% of health facilities
- ☒ Not provided in any health facility
- ☐ Don't know
-

5. Community leadership

- Ensure that community-led organizations deliver 30% of testing and treatment services by 2025, with a focus on HIV testing, linkage to treatment, adherence and retention support, and treatment literacy.
- Ensure that community-led organizations deliver 80% of HIV prevention services for populations at high risk of HIV infection by 2025, including for women within those populations.
- Ensure that community-led organizations deliver 60% of programmes to support the achievement of societal enablers by 2025.

89. Does your country differentiate between community-led organizations and other types of civil society organizations in the national HIV strategic plan, community health strategies or other documents guiding the HIV response?

☒ Yes

☐ No

89.1 If yes, are the specific roles of community-led organizations articulated in the(se) document(s)?

☒ Yes

☐ No

90. Are there any laws, regulations or policies that provide for the registration of community-led organizations in your country (select all that apply)?

☒ Registration of organizations led by people living with HIV is possible

☒ Registration of organizations led by sex workers is possible

☒ Registration of organizations led by gay men and other men who have sex with men is possible

☒ Registration of organizations led by transgender people is possible

☒ Registration of organizations led by people who inject drugs is possible

☐ There are no laws, regulations or policies that provide for the registration of community-led organizations in the country

☐ Other (please specify): _____

91. Are there laws, policies or regulations that enable access to funding for community-led organizations (select all that apply)?

☐ Social contracting allowing for funding of service delivery by communities from domestic funding

☐ From international donors

☐ Require a certain percentage of government funding for community-led organizations to be allowed to operate

☒ No laws enabling access to funding, but community-led organizations are able to access funding under general laws, policies or regulations

☐ There are no laws, policies or regulations enabling access to funding for community-led organizations

☐ Other (please specify): _____

92. Are representatives of community-led organizations included in the national HIV coordinating mechanism or equivalent?

☐ There is no national HIV coordinating mechanism or equivalent

☐ There is a national HIV coordinating mechanism or equivalent, but it does not include any representatives of community-led organizations

☒ There is a national HIV coordinating mechanism or equivalent, and it includes representatives of community-led organizations

92.1 If yes, please specify from which of the following community-led organizations (including youth-led and women-led) are representatives included (select all that apply):

☒ Representatives of organizations led by people living with HIV

☐ Representatives of sex worker-led organizations

☒ Representatives of organizations led by gay men and other men who have sex with men

☒ Representatives of transgender-led organizations

☐ Representatives of organizations led by people who inject drugs

93. Can community-led organizations legally provide any of the following services (select all that apply)?

☒ Linkage to HIV treatment

☒ Adherence and retention support

☒ Treatment literacy

☒ Distribution of antiretroviral medicines

☒ Distribution of condoms and lubricants

☒ HIV testing

☐ Needle-syringe distribution

☐ Naloxone distribution

☒ Legal literacy

☒ Legal services

☒ Information on life skills-based HIV and sexuality education*

☒ Sexual and gender-based violence prevention, psychosocial and medical support and referrals

☒ Information on sexual and reproductive health

☒ Trainings for healthcare workers

Participation

94. Do people living with HIV participate* in developing national policies, guidelines and/or strategies relating to their health in your country?
☒ Yes
☐ No
95. Do women living with HIV participate* in developing national policies, guidelines and strategies relating to prevention of vertical transmission?
☒ Yes
☐ No
96. Do gay men and other men who have sex with men participate* in developing national policies, guidelines and/or strategies relating to their health in your country?
☒ Yes
☐ No
97. Do sex workers participate* in developing national policies, guidelines and strategies relating to their health in your country?
☒ Yes
☐ No
98. Do people who inject drugs participate* in developing national policies, guidelines and strategies relating to their health in your country?
☐ Yes
☒ No
99. Do transgender people participate* in developing national policies, guidelines and strategies relating to their health in your country?
☒ Yes
☐ No
100. Do former or current people in prisons and other closed settings participate* in developing national policies, guidelines and strategies relating to their health in your country?
☐ Yes
☒ No
101. Do young people (aged 15–24 years) participate* in developing national policies, guidelines and strategies relating to their health in your country?
☒ Yes
☐ No
- 101.1 If yes, do young people participate* in any of the following decision-making spaces in the national HIV response (where they exist)?

Decision-making space	Does it exist?	Do young people participate in this space?
Technical teams for the development, review and update of national AIDS strategies and plans	<u>YES</u>	<u>YES</u>
Technical teams for the development or review of programmes that relate to young people's access to HIV testing, treatment, care and support services	<u>YES</u>	<u>YES</u>
National AIDS Coordinating Authority or equivalent, with a broad-based multisector mandate	<u>NO</u>	<u>NO</u>
Global Fund to Fight AIDS, Tuberculosis and Malaria (Global Fund) Country Coordinating Mechanism	<u>YES</u>	<u>YES</u>
Community advisory body for hospitals, clinics and/or research projects	<u>NO</u>	<u>NO</u>
Other (please specify): _____	Yes/No	Yes/No

6. Realize human rights and eliminate stigma and discrimination

- Ensure that less than 10% of countries have restrictive legal and policy frameworks that unfairly target people living with, at risk of and affected by HIV by 2025. Examples include age of consent laws and laws related to HIV nondisclosure, exposure and transmission, laws that impose HIV-related travel restrictions and mandatory testing, and laws that lead to the denial or limitation of access to services by 2025.
- Invest US\$ 3.1 billion in societal enablers—including protection of human rights, reduction of stigma and discrimination and law reform, where appropriate—in low- and middle-income countries by 2025.
- Ensure that less than 10% of people living with, at risk of and affected by HIV experience stigma and discrimination by 2025.

Punitive laws

102. If HIV non-disclosure, exposure or transmission are criminalized, have any legal actions to decriminalize HIV non-disclosure, exposure or transmission either started or been in process in the last two years? Please select all that apply.

- ☐ Strategic litigation
- ☐ Proposal put before parliament
- ☐ Other (please specify): _____
- ☒ There are no laws at the national or subnational level criminalizing HIV non-disclosure, exposure or transmission

103. If transgender people are criminalized, have any legal actions to decriminalize transgender people either started or been in process in the last two years? Please select all that apply.

- ☐ Strategic litigation
- ☐ Proposal put before parliament
- ☐ Other (please specify): _____
- ☒ There are no laws at the national or subnational level that criminalize transgender people

104. If sex work is criminalized, have any legal actions to decriminalize sex work either started or been in process in the last two years? Please select all that apply.

- ☐ Strategic litigation
- ☐ Proposal put before parliament
- ☒ Other (please specify): **No legal actions taken or in process to decriminalize sex work**
- ☐ There are no laws at the national or subnational level criminalizing sex work

105. If same-sex sexual acts are criminalized, have any legal actions to decriminalize same-sex sexual acts either started or been in process in the last two years? Please select all that apply.

- ☐ Strategic litigation
- ☐ Proposal put before parliament
- ☒ Other (please specify): **No legal actions taken or in process to decriminalize same-sex sexual acts**
- ☐ There are no laws at the national or subnational level criminalizing same-sex sexual acts in private

106. If drug use and/or possession for personal use are criminal offences, have any legal actions to decriminalize drug use or possession for personal use either started or been in process in the last two years? Please select all that apply.

- ☐ Strategic litigation
 - ☐ Proposal put before parliament
 - ☒ Other (please specify): **No legal actions taken or in process to decriminalize drug use and/or possession for personal use**
 - ☐ There are no laws at the national or subnational level criminalizing drug use and/or possession for personal use
-

7. Universal health coverage and integration

- Invest in robust, resilient, equitable and publicly funded systems for health and social protection systems that provide 90% of people living with, at risk of and affected by HIV with people-centred and context-specific integrated services for: HIV and other communicable diseases; noncommunicable diseases; sexual and reproductive health care; gender-based violence; mental health; palliative care; treatment of alcohol dependence; drug use legal services; and other services they need for their overall health and well-being. Ensure that by 2025, 45% of people living with, at risk of and affected by HIV and AIDS have access to social protection benefits.
- Ensure that 90% of people in humanitarian settings have access to integrated HIV services.
- Ensure the systematic engagement of HIV responses in pandemic response infrastructure and arrangements, leveraging national HIV strategic plans to guide key elements of pandemic preparedness planning and ensuring that 95% of people living with, at risk of and affected by HIV are protected against pandemics, including COVID-19.

Cervical cancer

107. Have the recommendations in the 2021 World Health Organization (WHO) guidelines for screening of cervical pre-cancer lesions for cervical cancer prevention among women living with HIV been adopted in your country's national guidelines?



Yes



No

107.1 If yes, please indicate the WHO recommendations for women living with HIV that have been adopted (select all that apply):



Age to start regular cervical cancer screening is 25 years



HPV-DNA Is used as the primary screening test as part of a screen and treat approach



VIA is used as a primary screening test as part of a screen, triage and treat approach



Cytology Is used as a primary screening test as part of a screen and treat approach



Cytology Is used as a primary screening test as part of a screen, triage and treat approach



Screening interval recommended for cytology/VIA is every 3 years



Screening interval recommended for HPV-DNA is every 3–5 years



Other (please specify): _____

108. Has re-testing of women living with HIV who underwent treatment for cervical precancerous lesions been adopted in national guidelines (select all that apply):



Yes, at 12 months



Yes, at 24 months



Yes, using HPV-DNA



Yes, using VIA



Yes, using cytology

Sexually transmitted infections

109. Does your country have a national strategy or action plan for the prevention and control of sexually transmitted infections?



Yes



No

109.1 If yes, in what year were they last updated?



Before 2021



2021



2022



2023



2024

110. Does your country have national case management guidelines for sexually transmitted infections?



Yes



No

110.1 If yes, in what year were they last updated?



Before 2021



2021



2022



2023



2024

111. What are the recommended first line treatment for uncomplicated anogenital infection with *Neisseria gonorrhoeae* in adults in the most recent national treatment guideline (select all that apply)?

- ☐ Cefixime 400 mg orally
- ☐ Cefixime 400 mg orally PLUS azithromycin 1 g orally
- ☐ Cefixime 800 mg orally PLUS azithromycin 1 g orally
- ☐ Cefixime 800 mg orally PLUS azithromycin 2 g orally
- ☐ Ceftriaxone 250 mg intramuscularly
- ☐ Ceftriaxone 1 g intramuscularly
- ☒ Ceftriaxone 250 mg intramuscularly PLUS azithromycin 1 g orally
- ☒ Other (please specify): **Syndromic management used in the treatment for urethral discharge**

111.1 When were the treatment guidelines for *N. gonorrhoeae* last updated?

- ☐ Before 2022
- ☒ 2022
- ☐ 2023
- ☐ 2024
- ☐ Currently being updated
- ☐ No treatment guidelines

112. Is gonococcal antimicrobial resistance testing available in country?

- ☒ Yes, for clinical cases (i.e. to test in case of clinical treatment failure)
 - ☐ Yes, for annual surveillance of resistance
 - ☐ Yes, for clinical cases and annual surveillance of resistance
 - ☐ No
-

Social protection

113. ☒ Does the country have an approved social protection* strategy, policy or framework?
☐ Yes, and it is being implemented
☐ Yes, but it is not being implemented
☐ No
- 113.1 If yes:
113.1.a ☒ Does it refer to HIV?
☐ Yes
☐ No
- 113.1.b ☒ Does it recognize people living with HIV as key beneficiaries?
☐ Yes
☐ No
- 113.1.b.i. If no, please describe any conditions under which people living with HIV can access social protection benefits: **Social Assessment**
- 113.1.c ☒ Does it recognize any key populations (sex workers, gay men and other men who have sex with men, people who inject drugs, transgender people or people in prisons and other closed settings) as key beneficiaries?
☐ Yes
☐ No
- 113.1.c.i. If yes, which key populations are recognized as key beneficiaries (select all that apply)?
☐ Sex workers
☐ Gay men and other men who have sex with men
☐ Transgender people
☐ People who inject drugs
☐ People in prisons and other closed settings
- 113.1.c.ii. If no, please describe any conditions under which key populations can access social protection benefits: **Social Assessment**
- 113.1.d ☒ Does it recognize adolescent girls and young women as key beneficiaries?
☐ Yes
☐ No
- 113.1.e ☒ Does it recognize children affected by HIV as key beneficiaries?
☐ Yes
☐ No
- 113.1.f ☒ Does it recognize families affected by HIV as key beneficiaries?
☐ Yes
☐ No
- 113.1.g ☒ Does it address the issue of unpaid care work in the context of HIV?
☐ Yes
☐ No
-
114. ☒ Are representatives of the National AIDS Programme or equivalent included in any social protection* coordination mechanism or platform?
☐ There is no social protection coordination mechanism or platform
☐ There is a social protection coordination mechanism or platform, but it does not include any representatives of the National AIDS Programme or equivalent
☐ There is a social protection coordination mechanism or platform, and it includes representatives of the National AIDS Programme or equivalent
-
115. ☒ Are any cash transfer* programmes for young women aged 15–24 years being implemented in the country?
☐ Yes
☐ No
-

8. Data, science and innovation

Information system

116. Are patient-level data routinely available within the health information system?

- ☐ Yes, fully electronic
- ☒ Yes, partially electronic
- ☐ Yes, paper-based only
- ☐ No health information system exists

116.1 If patient-level data exist, are antenatal care testing and treatment cascade data included in the health information system at the district level (select all that apply)?

- ☒ Pregnant women tested during antenatal care and outcome of test
- ☒ Women already on antiretroviral therapy at first antenatal care visit
- ☒ The number of people tested for HIV
- ☒ The number of people testing HIV-positive
- ☒ The number of people newly diagnosed with HIV who are on antiretroviral therapy
- ☒ The number of people on antiretroviral therapy who are virally suppressed

117. Are data from community-led organizations on services they delivered integrated in the national health information system or equivalent?

- ☒ Yes, community-led organizations are included as a type of provider for disaggregation of existing indicators
- ☐ Yes, specific indicators on service delivery by community-led organizations are included
- ☐ No
- ☐ Community-led organizations cannot provide services

Surveillance

118. Is HIV a nationally notifiable condition by law?

- ☒ Yes
- ☐ No

119. Does the country have a national HIV case surveillance* system?

- ☒ Yes
- ☐ No

119.1 If yes, does the national HIV case surveillance system include the following (select all that apply)?

- ☒ Individual-level data for each person diagnosed with HIV
- ☒ Collection of data from different sources (laboratories, testing and treatment records) to promote completeness of data on HIV case
- ☒ Linkage of individual-level data to remove duplicate records
- ☒ CD4 count at HIV diagnosis
- ☒ Initiation of antiretroviral therapy
- ☒ First and follow-up viral load test results
- ☒ Pregnancy in women living with HIV
- ☒ Death

Patient monitoring systems

120. Has the country updated the HIV testing and treatment indicators and tools using the 2022 World Health Organization (WHO) Consolidated guidelines on person-centered HIV strategic information?

- ☐ Yes, fully
- ☒ Yes, partially
- ☐ No
- ☐ Don't know

Unique identification codes for patients

121. Does the country have a method to identify and remove duplicate health information for patients within and between clinics —such as linking records using unique identifiers and/or personal identifiable information (including biometrics)—for the following services?

	Method to identify and remove duplicate health information	If yes, please specify how data are linked
Treatment services	☒ Yes, nationally harmonized ☐ Yes, but varies across regions ☐ Yes, but varies across programmes ☐ No ☐ Don't know	☒ National unique personal identifier ☒ HIV-specific unique identifier ☒ Combination of routinely collected personal identifying information ☐ Biometric ☐ Other (please specify) _____
Testing services	☐ Yes, nationally harmonized ☐ Yes, but varies across regions ☒ Yes, but varies across programmes ☐ No ☐ Don't know	☒ National unique personal identifier ☐ HIV-specific unique identifier ☒ Combination of routinely collected personal identifying information ☐ Biometric ☐ Other (please specify) _____
Laboratory services	☒ Yes, nationally harmonized ☐ Yes, but varies across regions ☐ Yes, but varies across programmes ☐ No ☐ Don't know	☒ National unique personal identifier ☐ HIV-specific unique identifier ☐ Combination of routinely collected personal identifying information ☐ Biometric ☒ Other (please specify) Medical Facility Unique Identifier

HIV prevention services designed for any key population group to track combination prevention uptake

Gay men and other men who have sex with men	☒ Yes, nationally harmonized ☐ Yes, but varies across regions ☐ Yes, but varies across programmes ☐ No ☐ Don't know	☒ National unique personal identifier ☐ HIV-specific unique identifier ☒ Combination of routinely collected personal identifying information ☐ Biometric ☐ Other (please specify) _____
Sex workers	☒ Yes, nationally harmonized ☐ Yes, but varies across regions ☐ Yes, but varies across programmes ☐ No ☐ Don't know	☒ National unique personal identifier ☐ HIV-specific unique identifier ☒ Combination of routinely collected personal identifying information ☐ Biometric ☐ Other (please specify) _____
Transgender people	☒ Yes, nationally harmonized ☐ Yes, but varies across regions ☐ Yes, but varies across programmes ☐ No ☐ Don't know	☒ National unique personal identifier ☐ HIV-specific unique identifier ☒ Combination of routinely collected personal identifying information ☐ Biometric ☐ Other (please specify) _____
People who inject drugs	☒ Yes, nationally harmonized ☐ Yes, but varies across regions ☐ Yes, but varies across programmes ☐ No ☐ Don't know	☒ National unique personal identifier ☐ HIV-specific unique identifier ☒ Combination of routinely collected personal identifying information ☐ Biometric ☐ Other (please specify) _____
Other (please specify): _____	☐ Yes, nationally harmonized ☐ Yes, but varies across regions ☐ Yes, but varies across programmes ☐ No ☐ Don't know	☐ National unique personal identifier ☐ HIV-specific unique identifier ☐ Combination of routinely collected personal identifying information ☐ Biometric ☐ Other (please specify) _____

121.1 If yes to any of the above, does the unique identifier policy also provide for legally enforceable data privacy protections?

☒ Yes
☐ No

122. When was the most recent data quality review conducted to determine the accuracy of national-level numbers of people reported to be on treatment?

- ☐ Currently being conducted (results expected next year)
- ☒ Completed in the last year and results available
- ☐ Completed 2 to 5 years ago
- ☐ Never conducted or conducted more than 5 years ago

122.1 If a data quality review has been conducted in the last year, have the results been used to adjust the numbers of people on treatment reported in Indicator 2.2?

- ☐ Yes
- ☒ No

123. When was the most recent data quality review conducted to determine the accuracy of the number of people reported to have suppressed viral loads?

- ☐ Currently being conducted (results expected next year)
- ☒ Completed in the last year and results available
- ☐ Completed 2 to 5 years ago
- ☐ Never conducted or conducted more than 5 years ago

123.1 If a data quality review has been conducted in the last year, has this been used to adjust the number of people who have suppressed viral loads reported in Indicator 2.3?

- ☐ Yes
- ☒ No

Data use

124. Are data reviews of HIV treatment cascade data being conducted?

- ☒ Yes
- ☐ No

124.1 If yes, please select the frequency with which data reviews of HIV treatment cascade data are conducted.

- ☒ Quarterly
- ☐ Every 6 months
- ☐ Annually
- ☐ Other, please specify: _____

124.2 If yes, at which level are data reviews conducted (select all that apply)?

- ☒ National
- ☒ District
- ☒ Facility